

Emis

Congenital defects??and medications /substance misuse in the pregnancy!

Sodium Valproate

~~Diazepam~~

Carbamazepine

Opioids

Smoking

1-10 year old with cleft palate and delayed milestones (Valproate)

2-Microcephaly-challenging behavior and poor memory, 2kg at birth (alcohol)

3- Jittery child, overarousal 2 hrs after delivery- cocaine- heroin-smoking

3-Which of the following would you do (new drug) initially for side effects of a drug for 3 people?

Case series

Case control

N of 1

Cohort

4- Which study to see the association between depression and lithium level

Case control/ cohort/ ecological/cross sectional (type of study)

5- Study for Effectiveness of Trainee Gp in managing medical issues for pt with schizo –

Cluster/ case control/ service evaluation/ Qualitative

6- Transmission of Down syndrome transmitted equally by both parents?

Robertsonian translocation (inherited)

Trisomy(sporadic)

Mosaicism (sporadic)

7-2 Radiologists for MRI results- Concurrent validity /Content validity/ inter rater validity (mock EMI)

8- New test for suicide, was seen that those who scored higher were more like to commit suicide- Predictive validity / Concurrent validity /Content validity/ inter rater validity

9-Galbraith plot is used for – measuring publication bias/heterogeneity/(L.ABBE, FUNNEL)

10- In regards to the following which one is true- gailbrath plot and forest plot can both be used- to check heterogeneity

11- bournewood case- Liberty / Gillick /Tarasoff/Bolam()/ Prichard/Negligence doc

12- Which of the following drugs should you avoid with Tamoxifen? Amitriptyline, Fluoxetine.

13- Block Randomisation is used for: ensuring equal no. of pts in each arm/ confounders

14- 40+ yr old woman who has completed detox for alcohol, now wants a drug to maintain abstinence. She is on Tramadol for chronic back pain and had history of ischemic heart disease. Which drug would you choose

Naltrexone/ **acamprosate**/diazepam/disulfiram

15- Drug with effect on GABA and NMDA antagonist- multiple uses –

Ketamine/cocaine/cannabis/LSD

16- Homeless man found with confusion, unstable gait and double vision: what would you give him first- Glucose/ **Thiamine**/

17- Female, partner left, “chronic back pain, now feeling helpless with pain- **somatoform pain** /cfs/depression/fibromyalgia

18- Pt with muscle pain, episodic low mood, palpitation, motor tension, muscle aches- **GAD**/cfs/depression/fibromyalgia/~~panic disorder~~

Viral infection, c/o pain in body- **cfs**/depression/**fibromyalgia**

19- Young man feeling low, experimented with drugs and presented with 2 episodes of **seizures**, sweating, **muscular rigidity**, tachycardia-

Amphetamines/**Phencyclidine**/Cocaine/Phencyclidine /Ketamine.

20- New born baby- jittery, shaking – cocaine/**Heroin**?/alcohol

21- Person with epilepsy brought to A&E with carbamazepine induced hyponatremia, also has depression. Which antidepressant would you choose

Amitriptyline

Sertraline

Citalopram

Mirtazapine

Venlafaxine

22- Patient presented with memory issues since 8 months- Gp suspected Alzheimer which of the following would make you consider an alternative diagnosis? **Urinary Incontinence**/

Preserved long term memory/ **apathy**/ forgot location..

23- Which of the following cannot be a Comorbid diagnosis with recurrent depressive disorder? Dysthymia/Cyclothymia/EUPD/schizoaffective/**adjustment**

24-ECGs findings

T wave , Saddle shaped ST segment (**MI... Pericarditis... Clozapine**)

Sinus rhythm with increased rate

Sinus rhythm with decreased rate

Low voltage QRS, other morphologies

Tall tented P waves

- a- Lady with Bulimia
- b- person recently started on clozapine reports chest pain

25- 80 year old woman recently started on donepezil presented with ankle swelling
Donepezil 10mg – gastrointestinal side effect- **reduce dose** /change to galantamine/add pantoprazole

26- 80 year old woman needs an antipsychotic, eGfr >25 ml,
Chlorpromazine/**Olanzapine**/**Haloperidol**/amisulpride/sulrpide

27- 14 year old female took overdose of paracetamol after fight with BF, presented with red face, tongue, confusion, mmse- 21/30, which of the following can explain the presentation:
Acute stress disorder/ **Delirium**/ adjustment disorder /drug induced psychosis

28- Which of the following gait will you see in an upper motor Neuron lesion: High steppage/ Magnetic gait/stiff leg **Scissoring gait** /antalgic gait

29- CEAR- curve -Esteketime improved Qualy 0.07 and costed 16,6xx in community and 16,9xx in hospital- which is true:
Esketamine was more expensive than the antidepressant and not effective
?We will need to do an CEAR curve to determine WTP before making a decision.?
We need a CEAC plane to determine.

30- Which describes the outcome in units of Qualy-**Cost Utility** / cost consequence/ cost benefit/

31- Mother come to visit 24 yr him and he becomes agitated. Every weekWhat is the best way to make him understand about the visits? CBT/**social stories(for autism)**/explaining the reason of visit, meds for calming/**functional behaviour analysis.**

Patient finds it funny when others imitate his behavior/tech used to assure the patient..?- social stories/**intensive interaction techniques**/Makaton(sign language)/now and then cards/

The schedule is fixed LD what would you use to tell about what is happening now and next- Makaton(sign language)/**now and then cards**/

32- Best predictor of violence- **Previous violence attempts**/ alcohol and drugs /possession of firearms.

33- The majority of the violent acts are due to:
Alcohol and drugs/ personality disorders /

34- Amnesia after violent offence – **seen in crimes of passion**(too much emotions, in hyperarousal) /often resolves after the trial is over/ often seen in malingering.

35- Post stroke incidence, due to antipsychotics-? 0.1?/ 0.01%/ 3% /1%/10%

36- A woman kills her baby due to psychosis 'end the misery' of the sick child- this is diminished responsibility/ insanity/

37- Cost benefit analysis refers to- Reducing the cost for the same outcome

38- What is ICER- The cost of intervention compared to control for same outcome/ Ratio between cost and benefit in terms of quality of life.

39- Patient with severe self harm behaviour- Lesch Nyhan syndrome (other disorders are Prader willi, cri du chat, cornelia, smyth magnis)

40- Agomelatine study—(fresh question 2021)

The strength of the study – unpublished studies

Conclusion- It may be a useful antidepressant/ it works well as an adjunct antidepressant

Where will you find the literature- medline, google scholar, psychinfo, New opt.. (don't rem)

41- ECT Forest plot – (mock 13,fresh quest 2021/fresh qs set 6)

Best Treatment in relation to benefit and tolerance _Priming

Treatment with the most precise – BT (left)

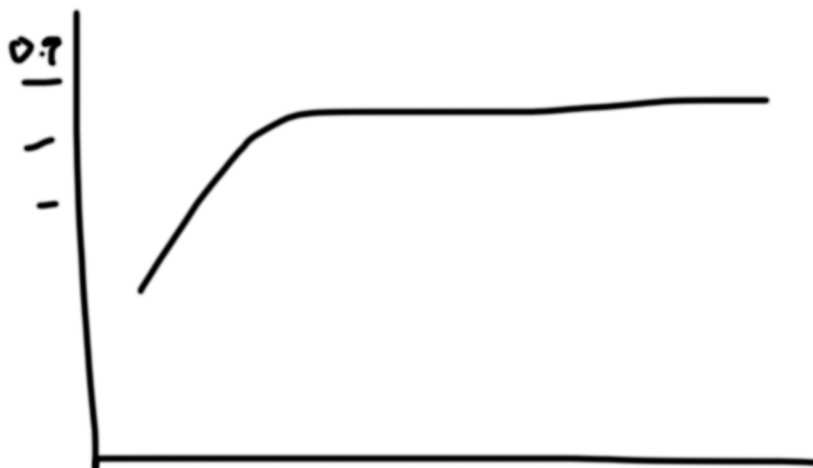
42- Cannabis usage-

Odds ratio of those using it daily (user- 410, psycho 123= Control- 310, 41) 2.5/ 3.4/ 0.3

Psychosis was most related to – frequency of usage/ duration.

Odds Ratio= $a/b/c/d = ad/bc$

43- ICER curve (fresh question 2021)



Which of the following regarding SD is true:

There was >50% increment in the effectiveness as compared to the community.

Cannot be commented. ..

Cannot determine the efficacy from the ICER curve...

44- Funnel plot (Fresh question 2021)

– what would be the effect if the studies were not imputed?

The smaller negative studies would be missed--- plot asymmetrical

Reason for imputation- more representative/type 2 error/reduce publication, want same no. of studies on both sides of line of no effect.

45- Consort diagram for CBT in prenatal : (The loss to follow 46 in R arm & 44 in L arm) The same no. were imputed in the primary analysis.

What was the reason for imputation in the study? Representation of the Missing data.

46- family therapies

Eclectic... (improve communication between family members.)

Systemic

Strategic

Structural

Psychodynamic

1- Family functioning on previous behaviors (psychodynamic F.T)

2- Paradoxical injunctions(Systemic)

3- Unsaid rules (Structural)

47- Female presented to A&E sudden aphonia after mother coming to live with them- Motor dissociative disorder/ dissociative seizures/ acute stress/ adjustment/somatization.

48- Pt on antipsychotics, non compliant, using amphetamines presented with mutism, catalepsy, fever-/not on antipsychotics- what is the diagnosis- Malignant catatonia/ Malignant hyperthermia/NMS/..(stimulants can also cause,, anesthetics.)

49- Pt with parietal lobe infarct, agitated and confused which of following would you give- Low dose Risperidone(risk of stroke) as needed trial, low dose haloperidol as needed, trial, lorazepam.

50-) 33 yr, Female with moderate LD and mania, which of the following would you not prescribe:

Lithium/ Carbamazepine/ Valproate/ Lamotrigine/ Levetiracetam

51- female in 30s, 3 dy hx of mutism and immobility- what would you do first?

ECT

Lorazepam

Haloperidol

Watchful waiting

52- Elderly male440 female470, presented with QTC of 490ms on sertraline 50 and risp 2mg, next step:

Reduce Risperidone

Switch to Quetiapine
Repeat ECG next week
Refer to cardiologist urgently

53- Woman on Citalopram 40mg for 2 months, no response what next?

Switch to another SSRI

Augment with lithium
Increase the dose.

54- Stat tests (3

- 1- Non parametric -mann whitney U.
- 2- ANCOVA /anova 2 v- baseline at 3month/6 month/ 12 month/ (HAMD)
- 3- T test

Chi sq/Linear regression/t test/

55-difference btw the mean of Mirtazapine and placebo group at 12 weeks.

Variables	Mirtazapine + SSRI/ Nasra		Placebo + SSRI / SNRI	
	Number	mean (SD)	Number	mean (SD)
Baseline	241	31.5 (10.2)	239	30.6 (9.6)
Primary outcome After 12 weeks	214	18 (12.3)	217	19.7 (12.4)

55- MCI to Dementia, daughters wants to know how to reduce his risk- reduce smoking, aspirin(if co morbidities), **cannot do anything**/omega 3/ vit B rich /zinc

56- which of the following is unethical for cannabis and psychosis – case control/
RCT/cohort/N of 1

57- **prevalence** throughout European countries – socioeconomic levels and depression–
study design- ecological/ **cross sectional**/

58-

59- Life increase, total cases were increasing ,new cases of dementia were slowly reducing-
true- prevalence going down, incidence reducing slowly, prevalence same, **high prevalence**

60- Audit – **implementing the change wasn't done**. (repeat) fresh questions..

61-you wish to get views of mdt members about role of psychiatrist= **purposive sampling**/ triangulation/ reflexivity/

62- Choose 2 different views for --**Triangulation**/ Reflexivity/ grounded theory/

Researcher's views/opinions – **reflexivity**,

63-Analysis

??64- Histogram- (connected bars for continuous data.)

Skew can be seen

Width

65- time to hospitalization for ALz + LBD – early hospitalization than ALz/ more at home ALz than LBD. / at the end were significantly different/ (**Kaplan Meier Curve**)

66- significant reason for lower life expectancy in schizo- suicide/copd/**CVD**/DM(dies 10 years early)

67- toxicity of Lithium – polyuria/ abd pain/ fine tremor /**myoclonic twitches** >1.2 mg

68- pt with EPSE- which would you see- paramyoclonic clasp knife rigidity(Extrapyramidal=parkinson) /intentional tremor/**bradykinesia**

69- Post-partum, intrusive recurring thoughts, distress causing- traumatic birth perceived by mom- 1 year baby- **OCD**

70- Tool to measure loneliness 0.38 and depression what does this indicate: 0.38 linear regression-

71- (F=1.34, CI- 1.02-1.5) what does that F mean- adjusted effect size/unadjusted effect size/variance. (**ratio of two variances is F- statistic**) (Df degree of freedom)

72- refractory OCD- **Clomipramine** and Citalopram/ Fluoxetine/ Clomipramine and Prozac/

73- NNT- 9 =1/ARR (EER-CER)

74- BDI- (mean 1.7) ? 21 self rating/ 14years up for depression screening. **See Q45 Fresh questions 22**

??75- Since 12 weeks on Risperidone 1mg BD and citalopram 20mg 2 days ago.. - options increasing to 1.5 once, increase citalopram/ **reduce risperidone**/add lorazepam 1mg or reg.

??76- Key role for CBT in psychosis

Acceptance/ Normalization/decreasing stress

Validation

Reattribution

Distress tolerance

77- phone call for which of the following therapy:

DBT/

??78- Which of the following do you hierarchy of anxiety

ERP/Desensitisation/CBT/

79- Motivational interview – rolling with resistance/avoidance resistance/developing discrepancy/amplifying cognitive dissonance in which therapy/incentives. (contingency management).

80 alcohol reduction – increasing the price per unit /social awareness.

81- prevalence of antisocial in prison- 50%

??82- You see remission of EUPD after diagnosis of 10 years= <10%, 20-30%(30-50%), 50%, 86%

83- Which of the disorders increases in severity and prevalence btw 10-12?

ASD/Tourettes/OCD/ADHD.

84- ASD- prevalence – heritability 90%/ 40% normal in intelligent/LD upto 70%

85- Suicide in prison- more common in females/life time sentenced /vs pt in remanded /physical health issues

86-International suicides- males more than females/suicides in prison are more than gen pop/overcrowded prisons/lower education of staff.

87- Increased risk of psychosis, 20-30% more common in- Fragile X, (di George syndrome)Velocardiofacial, William

88- Equal male to female prevalence- prader willi- fragile x -

89- Schizophrenia in child and adolescent- Hospital admission is preferred for MDT assessment, 1% prevalence, incidence higher after the age 15/ 0.5% PREVELANCE

90- Sleep terror vs nightmare- not recalling the episode(terror),

91- Narcolepsy- sleep paralysis- catalepsy- extracampine hallucination- cataplexy

92- melatonin in children, what will be seen- decrease sleep latency/sleep duration/ sleep architecture/no.of wakening.

??93- Theft – substance use and alcohol/personality disorder/impulsive pathological

94- ITT_ done to- give accurate results despite patient's nonadherence to protocol./ prevent type 2 error/**more type 1 error**

95- Seq diagram formulation(visual flow chart) used in – **CAT**
3 Rs= reformulation, recognition, revision)

96- conditioning in OCD= **negative reinforcement**. /reciprocal inhibition/classic conditional/

97- study, avoiding attrition for finding a risk btw xxx- **case control**(less attrition bias)/ cohort(less recall bias)/

98- Clozapine not given- **paralytic ileus**/ HTN

99- Cutlass results- second generation have lesser side effects/ **except clozapine, other antipsychotic equally effective**/ Olanzapine had the highest tendency to gain weight/ clozapine was more effective.

100- therapist gives interpretation and results – **neg therapeutic reaction**.

101- pt with schizophrenia- active psychotic sym- which test would be affected- selective attention/ **ROCF..Check both working and executive functioning..Ray ostrichit**/ Clock drawing(executive function)

102- Clonidine in ADHD is given – what does it not improve- **aggression/ attention/impulsivity/**

103- Simple tic example would be – throat clearing/ grunting/ grimacing
SIMPLE MOTOR TIC: head shaking, eye blinking, sniffing, neck jerking, shoulder shrugging, grimacing.
Simple vocal tics: coughing, throat clearing,grunting, barking,

104- Roc accuracy --- AUC 0.86 .. sensitivity 100%
x axis= 1-specificity
y axis: sensitivity

105- Mild euphoria, giggly, relaxed and this is usually smoked- **cannabis**/nicotine/

106- Choosing the balance of participants based on initial selection people- **minimization**
Participants based on age group= matching/stratified randomisation

? 107- Short acting stimulant and full agonist- heroin/ codeine/ buprenorphine /synthetic cannabinoid/cannabis.

108- 10 year old mild depression- digital Cbt/ group CBT/ **watchful waiting**/ medication.

109- Child with PTSD- talk to him apart from parents directly about PTSD sx/ **Avoid talking till the child talks**/ early debriefing

110- ASPD in prison- 50% or 70%

111- Study on LD participants which is true- they can withdraw the consent anytime/ they cannot withdraw consent after signing

112- methadone 70ml, 10 week pregnant wants to stop, next step- reduce in 2 weeks, continue the same,

Recall March 2021

- **ECT** — Most Common memory affected — Retrograde and Anterograde no option
 - Implicit
 - **Explicit**
 - Procedural
 - Topographical
 - Working
- **ECT** — Number of people remit — 0.3; 0.5; 0.7-0.9 etc
- Relapse is as high as 84% within 1-6 month after ECT.
- Sample Mean ; how close is the sample mean true/ population mean — **Standard Error of Mean**
- **** P = 0.01, and CI = (very narrow with 0.02 difference e.g 0.5-0.7) — **Large Sample Size**
- **EMis** Treatment Resistant Condition - PTSD, GAD, Hypochondriasis, Social Phobia
 - SSRI + Buspirone — GAD
 - Pregablin when SNRI and SSRI fails GAD
 - Antipsychotic + Clomipramine to SSRI OCD
- Conduct Disorder Subtypes
 - Hyperactive
 - Non-social able
- Lady could not speak English + Translator + Cognitive Impairment
 - **MOCA (MONTREAL cognitive assessment)**
 - ACE
- HTN managed on Ramipril; ACE-3 was 87/100; diagnosis
 - MCI (82/100=? Dementia... > 87/100 fine... 82-87/100 mci)
-
- How will you diff MCI from Alzheimer's dementia
 - Clock Drawing Test
 - ?MMSE

- 2 stimulants tried still has symptoms — Methylphenidate; atomoxetine; risperidone; DAAM(Dexamphetamine)
- ID + Talks to herself; bad dreams; wakes up other residents + challenging behaviour — ? Risperidone
- Challenging behaviour + ID during periods + punches her tummy — ?

- Levonostrel (estrogen analogue)

(First line intermittent SSRI)

- Reboxetine
- Antipsychotic
- ??Ibuprofen

- Once a day drug for challenging behavior.
- Methylphenidate (quick release, IR) Methylphenidate XR / Modafinil / — in school teachers think once only will help to do activities during school (08 and 1600)

- LBD / PD Dementia

- CI + PD (6 months ago) + visual hallucinations 2 years history etc Tremor, rigidity + olanzapine —> rigidity etc (Short term memory loss + falls.. acting out in sleep.. was diagnosed with parkinsons 6months back.. what's the diagnosis?)

- MMSE 26/30 ; personality change ; sexual interest in brother in law in front of grand kids — FTD; Alzheimers; LBD; MCI

- Galantamine — mc effective in

- Alzheimer with Vascular?
- FTD?
- PDD
- MCI
- Mixed dementia



- Admitted to nursing care then + BPSD + strong adjective aggression; 2 weeks — ?Donepezil / ?continuous Risperidone



Old pt with disinhibition, behavioural symptoms.. what type of language abn seen first? = fluent expressive / non fluent expressive / receptive aphasia/verbigeration



- Choosing Statistical Tests

- Paired T-test — Pre and post BP measurement
- One way ANOVA — Average weight of patient across 10 hospitals
- Chi-square — ?Cross over trial
- Kruskal-Wallis — 3 grasps; consultants; trainers and one more group; ordinal data (likert scale)



- Logistic regression

- one dependent variable (dichotomous: autism vs no autism) and one or multiple independent —
- One dependent variable (weight gain) and multiple independent variables (antipsychotics, antidepressants and one more
- Multiple linear regression



- Risk of Cannabis — Psychosis — 2 times

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- ?Automatisms / Diminished REsponsibility— The person has no sense of his/her ac 
- Act Rea 

 Reducing  ion — Increase Staff Patient Ratio

- Reduce RE-offending in people released
 - Treat BPAD
 - Treat Psychosis
 - Treat Depression
 - Treat Opioid Dependence
 - Treat ADHD

 Culpable Jury has to be present 

- Diminished Responsibility — Mu 
- Mosaics and Downs — Normal IQ
- Retts — MECP2

 FAS — Macrocephaly, Tall Stature, Palpebral Fissure 

- More likely to re-offend
 - Sexual deviances with stranger
 - Porn of Computer
 - Deviant sexual interests with anti-social traits

• Daingosis in BPAD

- Irritability; less need for sleep; h/o of several depressive episodes; never affected his work — bipolar 2; cyclothymia;

 Bipolar 2? — male with previous history of increased engird sexual interest, followed by low engird and no interest

 Female on some typical antipsychotic depot & procyclidine since 20yrs.. now having ?tardive dyskinesia.. first step= reduce antipsychotic / change to atypical /s top procyclidine/ start tetrabenazine

• PRader Willi — 1:1

• EMIs 15 year

 Depression — ?High Cortisol

- Anorexia — ?family Enmeshment 

 Conduct — other options Behavioural activation; maternal smoking

• Prevention Level — EMIs (options different Prevention levels; Primary selective and primary indicative)

 Blood Test in child for PKU — 

• Rivastigmine in Dementia

• Security measures in many places to prevent suicide 

• Man goes to GP for antidepressant medications but he  s him to another psychological and social practice prescriber 

• 2 questions on Defence Immature ego defense/ Neuro  defenses

- Compensation

- **Idealisation**
- displacement
- **Intellectualisation**
- splitting
- sublimation



- 1 cognitive distortion

- **PERsonalisation**
- Other options with FTD

- Psychotherapy EMI
- circular questioning in — **Systemic Family Therapy**
- Reformuln recogn revision in — **CAT**



task setting in — **Systemic Family Therapy**

- Poly substance (Serotonin, Clonidine and **Haloperidol** + Fever + Hyper refexia + Clonus + 8 hours ago took — **Serotonin Syndrome**
- Psychosis with Diabetic retinopathy.. not responding to Arip.. what next to be tried? **Amisul** / Halop/Olanz etc
- Polysubstnace + Boy + Nystagmus + Slurred Speech + Incoordiantion — **Benzo Intox**; Opoird Intox; Amphetamine Intox; Benzo Withdrawl
- Staccto Speech — **Cerebellar Degeneration**
- OCD co-morbidly associated with



- Mood disorder
- Anankastic personality
- Schizoid personality
- **GAD**

- Synthetic Canniboids presents to A&E with **Agitation**; depression; bradycardia; HTN

- ADHD — pathway — **Frontostriatal** and other pathway as options
- OCD in Adolescent (Clomipramine, **Paroxetine**, **Fluvoxamine**, Aripiprazole)



Anorexia when to Admit — BMI 14; HR , 50-60 ; **can't stand up from lying** self harm

- lady on Lithium, 600mg delivery next week , next step- **reduced to 400** /increase to 800/ s
- Autism cannot see the whole picture — **Central Coherence Theory**, Hypersystematisation
- lady on Lithium, 600mg delivery next week , does not want to brestfeed, Level - 0.75 next st



- **reduced to 400 at delivery and repeat in 1 week**
- increase to 800
- Stop

- Mild Depression in Teenagers

- **Digital CBT**
- Family Therapy
- Watchful waiting 2 months
- SSRI



- University submission; reaches his brothers house, without knowledge of how he got there — **Dissociative Fugue**
- 30 year h/o of mixed anxiety and depression and tingling sensation, seizure + consciousness not impaired — **?dissociative convulsions**
- before exams starts with left hand then later, no movement of hand and spreads to leg, examination was normal — **dissociative motor something**
- Autonomic questions (sweating, urinary urge and palpitations, investigations — normal — Dissociative autonomic something; ? autonomic somatoform
- Non-Seizure vs epileptic Seizure — **dissociative can have a history of Epileptic as well**



Transactions meaning

- Understanding each other's ego state etc something
- Group - people feel others share their thoughts/feelings and problems- **Universality** (Yalon's curative factor)

• Transference

- Related to theprait only
- Should be ignored
- Psychotic phenomenon
- **It takes months for it to happen**
- **It is explained by mechanism of sublimation**

• Study of Kaplan (survival curve)

- Developing at 40 months (**25%**)
- RR between developing and placebo (**50 - 25%**)
- What does
-

• Pregablin and Gabapentinopid

• Case Control study and side-effect of a

- Odds Ratio
- **Hazard Ratio**

• Forest Plot — p value with 0.0 (g — effect size);

- Pooled effect have small to moderate effect / and (or but not) statistically significant
- How many were significant — **4 / 6**
- Which study had the biggest sample size

• Qualitative study

- Reflexivity
- Ethnography — Interaction between culture and groups something
- Thematic analysis — saturation
- Fixed Effect Analysis



- **Assume uncertainly not there**
- Assume error in within the study groups
- Assume every study has been a error value (something like that)

- Ecological Study
 - **Population**
- Ecological Fallacy
 - **Cannot make Individual assumptions**
- Something
 - Sampling analysis
 - Boot strapping
- Lady pouts when touched or  snout / pout reflex)
 - **Frontal Lobe** 
 - Other lobes
 - Cerebellum
- No psychological and physiological dependence (cannabis and LSD)
 - **LSD correct**
- Downs vs general population
 - Female has lesser IQ deficit than male
 - Development of Vasucular dementia (the correct answer for all but replaced with vascular)
 - **Language development is independent of IQ development**
- Question about schizophrenia
 - Onset same in male and females
 - **Father of affected child same as same s/e class as general population** 
 - ?Prevalence in developing country is more
 - Prevalence is low in low SE
 - Lifetime Risk is 5%
- Mother with schizophrenia — **baby with 13%**
- Mother on Paroxetine all throughout pregnancy
 - Facial Abormality
 - **Irritabitiy**
 - PPH
- Least like to be Modifiable Risk Factor for Dementia
 - Wilsons
 - Alcoholic
 - Chronic Subdural
 - Pellagra
 - **Motor Neuron Disease**
- STARD trail — First Antidepressant — **Citalopram**
- Alcohol withdrawal
 - **Unusual to stay after 5 days**

- Auditory hallucinations present
- Peaks in 4 days
- Seizure risk max in 3-5 days (not first day for sure)



- IV Groin Abscess + presents with abdominal cramps ; NEXT immediate action

- Buprenorphine
- Lofexidine
- Methadone
- Naloxone

- Factitious

- Does not allow invasive procedures
- Sharing info
- Less understanding of the illness
- Gives accurate symptomatology
- Numbers of visitors coming in

- Heroin user and wife gave ultimatum; he will get benzo stock and laxatives stock in 1 month (stock in 1 month) — Preparation
- Alcohol dependent told by GP that 14 units / per week and he has stayed in limit since 4 years — Maintenance
- Teen smokes and does not want to stop — Pre- contemplative

- Allocation concealment



- Highest Hierarchy of Evidence

- Cohort
- Cross Sectional
- Case Control
- Expert
- No meta and systemic

- CEAC

- What is the threshold; what will be the effectiveness
 - 0.6-0.8
 - 0.7-0.8
 - 0.4-0.6

- Cost benefits — outcomes measured in money terms
- Cost Minimisation — 2 services with equivalent results
- Blinding — Participant and clinical was blinded but not rater (what bias can he bring in)



- Reporting bias?



-
-
-
-